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NUTRITION COACH

Nutrition Fundamentals

What to eat, how much, and how to keep it simple enough to last

COMPLETE GUIDE 2 OF 5

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Disclaimer. This educational resource presents general nutrition information for healthy adults. It does not constitute medical, dietetic or therapeutic advice. The author is a nutrition coach, not a registered dietitian or physician. If you have a medical condition, take medication, are pregnant or breastfeeding, have a history of disordered eating, or have any clinical concern, consult a qualified professional before changing your diet or taking supplements. Individual results vary.

BEFORE YOU READ

Start here

Nutrition advice online is loud, contradictory and mostly written to sell you something. This guide does the opposite. It tells you what the evidence supports, how sure we are, and what to do with it **this week**.

The core message fits in one sentence: **the best nutrition plan is the one you can follow on an ordinary Tuesday, hit the numbers that matter, and repeat for months.** Everything else is decoration.

How to use this guide

- **Do the five numbers in Chapter 2 and 3 first.** Calories, protein, fat, carbs and one weekly check. That is 80% of the result.
- **Ignore the rest until those are stable.** Supplements and timing are the last 5%, not the first.
- **Use it with Guide 1.** Guide 1 (The TRANSFORM Method) tells you how to make a habit stick. This guide tells you which habits are worth sticking to.
- **If you have ADHD,** go straight to Chapter 9. The numbers are the same, the system around them is different.

I am a nutrition coach, not a doctor or a registered dietitian. This guide is educational, not medical advice. If you have a medical condition, take medication, are pregnant, or have ever had an eating disorder, read Chapter 14 before you change anything.

CHAPTER 1

What actually matters, in order

Think of nutrition as a pyramid. The bottom layers produce most of the result, and the top layers only matter once the bottom is solid. Most people spend their time at the top.

Layer	What it is	How much it matters
1	**Consistency.** Can you actually follow it for months?	The biggest factor. Nothing works if you quit.

2	**Energy balance.** Calories in versus calories out	Decides whether you lose, gain or maintain weight.
3	**Protein.** Enough, spread across the day	Protects muscle, keeps you full, supports training.
4	**Food quality.** Mostly whole, minimally processed food	Affects hunger, health and how easy layer 2 is.
5	**Carbs and fat for your training.** Fuel for your sport	Matters more the harder and longer you train.
6	**Meal timing and frequency**	Small. Useful for convenience and hunger.
7	**Supplements**	Smallest. A few have evidence, most do not.

Adherence beats the "perfect" diet

The strongest argument for starting with consistency comes from head-to-head trials. In a one-year randomized trial of 160 adults assigned to the Atkins, Zone, Weight Watchers or Ornish diets, **how much weight people lost was linked to how well they stuck with the diet (r = 0.60), not to which diet it was (r = 0.07).**[1] Adherence was low in every group, which is the real story: the diet that works is the one people keep doing.

The DIETFITS trial reached a similar conclusion with 609 adults randomized to a healthy low-fat or a healthy low-carbohydrate diet for 12 months. Average weight loss was 5.3 kg versus 6.0 kg, and the difference was not statistically significant. Neither genotype nor insulin response predicted who would do better on which diet.[2]

A broader look at tightly controlled feeding studies points the same way. A meta-analysis of 32 studies that swapped carbohydrate for fat at the same calories found only tiny differences: about 26 kcal per day more energy burned and about 16 g per day more fat lost on the **lower-fat** diets, the opposite of what low-carb advocates predicted.[3] Differences that small do not decide your results.

What this means for you

Stop searching for the best diet. Pick an approach that fits your food culture, budget and schedule, and put your effort into following it. Low carb, low fat, Mediterranean or flexible can all work if the calories and protein are right and you can stay with it.

CHAPTER 2

Energy balance: how much to eat

To lose weight you need to take in less energy than you use. To gain you need more. This is not controversial, but it is **more dynamic than the textbook version.** When you eat less, your body burns somewhat less too, and your weight settles at a new level over many months rather than falling in a straight line.[4]

That is why a calculator gives you a **starting point and not an answer.** You will adjust it with your own data in Chapter 10.

Step 1: estimate your maintenance

The Mifflin-St Jeor equation is the most reliable of the common formulas for estimating resting energy needs. A systematic review found it predicts within 10% of measured values more often than the alternatives, though it still misses for many individuals.[5][6]

	Formula (kg and cm)
Men	(10 x weight) + (6.25 x height) - (5 x age) + 5
Women	(10 x weight) + (6.25 x height) - (5 x age) - 161

Then multiply by an activity factor to estimate your total daily energy use. These multipliers are rough conventions, not measurements, so be honest and pick the **lower** one if you are unsure.

Activity	Multiplier
Mostly seated, little exercise	x 1.2
Light exercise 1 to 3 days per week	x 1.375
Moderate exercise 3 to 5 days per week	x 1.55
Hard training 6 to 7 days per week	x 1.725
Very hard training plus a physical job	x 1.9

Step 2: pick a deficit or a surplus

- **To lose fat:** eat about **300 to 500 kcal below** maintenance. A common target is losing roughly 0.5 to 1% of body weight per week, which lowers the risk of losing muscle.[7]
- **To build muscle:** eat about **200 to 300 kcal above** maintenance and train with resistance. Larger surpluses mostly add fat.
- **To maintain:** eat at your estimated maintenance and use the weekly check.

The "3,500 calorie rule" is wrong

You may have heard that a 3,500 kcal deficit equals one pound of fat lost, so 500 kcal a day gives a pound a week. Dynamic models show this overestimates long-term loss, because your energy needs drop as you lose weight.[4] Expect fast early progress (partly water), then a slower, steadier rate. That is not a stall, it is physiology.

A worked example

A 30 year old man, 80 kg, 175 cm, training four days a week. BMR = (10 × 80) + (6.25 × 175) - (5 x 30) + 5 = **1749 kcal**. Times 1.55 = about **2711 kcal** maintenance. For fat loss, subtract about 400: target roughly **2,300 kcal per day**.

Your turn

Write your own numbers in the worksheet at the end: BMR, maintenance, and your starting target. Round to the nearest 50. Do not agonize over the exact number, you will tune it.

CHAPTER 3

Protein: the one macro worth tracking first

If you only track one thing, track protein. It builds and repairs muscle, it fills you up more than carbohydrate or fat, and it costs the body more energy to digest.[8][9]

How much

A meta-analysis of 49 randomized trials with 1,863 participants found that the benefit of protein for gains in muscle plateaued at around **1.6 g per kg of body weight per day**, with the upper end of the confidence interval around 2.2 g per kg.[10] The ISSN position stand on protein and exercise lands in the same area for people who train, and calls 1.4 to 2.0 g per kg sufficient for most.[11]

Situation	Daily protein (per kg body weight)
General health, little training	0.8 to 1.2 g (a minimum, not an optimum)
Training regularly, maintaining or gaining	1.6 to 2.0 g
Fat loss while training	1.8 to 2.2 g
Very lean athlete in a large deficit	Possibly higher. A review suggests 2.3 to 3.1 g per kg of **lean mass**.[12]

This is a range, not a cliff. The difference between 1.8 and 2.2 g per kg is small. Hitting 1.6 consistently beats hitting 2.4 occasionally.

How to spread it

Eating protein at each meal is a practical way to reach your total and to control hunger. Research on muscle building suggests about **0.4 g per kg per meal** across at least four meals is a sound default, and that larger single doses are not wasted, just less efficient for muscle building at that moment.[13] For most people that means **25 to 45 g per meal.**

Protein and hunger

In a controlled study, when 19 people raised protein from 15% to 30% of calories (keeping carbohydrate constant), they spontaneously ate about 440 kcal less per day and lost weight without being told to restrict.[9] Reviews of higher-protein diets point the same way for appetite and weight control.[14] It is a small study, but it is one of the more practical ways to make a deficit easier, and the main reason to put protein first.

Food	Typical portion	Protein (approx.)
Chicken breast, cooked	150 g	45 g
Eggs	3 large	18 g

Greek yogurt, plain	200 g	20 g
Canned tuna	1 can, drained (140 g)	30 g
Lean beef, cooked	150 g	40 g
Tofu, firm	150 g	20 g
Lentils, cooked	1 cup (200 g)	18 g
Whey protein	1 scoop (30 g)	20 to 25 g

Values are approximate and vary by brand and cut. Check the label or a food database.

CHAPTER 4

Fat and carbohydrate: set them last

After calories and protein, the rest of your energy is split between fat and carbohydrate. Both have a floor, and above that floor the split is mostly **preference and training demands.**

Fat

Fat supports hormones, absorbs vitamins A, D, E and K, and makes food satisfying. The joint position statement of the Academy of Nutrition and Dietetics, Dietitians of Canada and the ACSM suggests about **20 to 35% of calories** from fat, and notes that going below 20% is not beneficial for performance.[15] A meta-analysis of six studies (206 men) also found that low-fat diets were associated with lower testosterone, though the authors say more randomized trials are needed.[16] A reasonable default is **about 25% of calories**, and not less than 20%.

Carbohydrate

Carbohydrate is the fuel that high-intensity training runs on, because it refills muscle glycogen.[17] The same position statement gives ranges by training load.[15]

Training load	Carbohydrate per kg per day
Light (low intensity or skill work)	3 to 5 g
Moderate (about 1 hour per day)	5 to 7 g

High (1 to 3 hours per day of moderate to hard)	6 to 10 g
Very high (4 to 5 hours per day)	8 to 12 g

These ranges are for athletes. If you are trying to lose fat, you will usually land at the low end, which is fine. If your training quality drops or you feel flat, that is a sign to raise carbohydrate before you cut anything else.

Putting the numbers together

Using the worked example from Chapter 2 (2,300 kcal, 80 kg):

Step	Calculation	Result
Protein	80 kg x 2.0 g = 160 g x 4 kcal	640 kcal
Fat	25% of 2,300 kcal = 575 kcal / 9	64 g
Carbohydrate	2,300 - 640 - 575 = 1085 kcal / 4	271 g

Daily target for this example

160 g protein, 64 g fat, 271 g carbohydrate, about 2,300 kcal. Ask yourself one question: "Can I eat this every day for three months?" If not, change the plan, not your character.

CHAPTER 5

Food quality and the 80/20 rule

Calories are the main driver of weight, but the **type of food changes how hard it is to control them.** The cleanest evidence comes from a tightly controlled trial at the US National Institutes of Health. Twenty adults lived in a metabolic ward and ate either an ultra-processed or an unprocessed diet for two weeks each, with meals matched for presented calories, sugar, fat, fiber and sodium. They could eat as much as they wanted. On the ultra-processed diet they ate about **508 kcal more per day** and gained 0.9 kg. On the unprocessed diet they lost 0.9 kg.[18]

It is a small, short study, so do not read too much into the exact numbers. But it supports a simple practical idea: **build most of your meals from foods that look like the thing they came from**, because they are easier to stop eating.

The 80/20 approach

- **About 80% of your food** comes from minimally processed sources: lean protein, vegetables, fruit, potatoes, rice, beans, oats, eggs, dairy, nuts and olive oil.
- **About 20% is for life:** the birthday cake, the pizza with friends, the thing you love. It counts toward your totals, and it does not count as failure.
- This is my coaching rule of thumb, not a measured threshold. What matters is the principle: **no food is banned.**

The reason to avoid bans has some evidence behind it. In a survey of 188 women, **rigid** dieting (all-or-nothing rules) was associated with more eating disorder symptoms, mood disturbance and higher BMI, while **flexible** dieting was not.[19] This was cross-sectional, so it cannot prove cause, but it matches what I see: people who treat one cookie as a disaster tend to turn it into a binge.

A simple plate for most meals

1. **Protein** takes about a quarter of the plate (a palm or two).
2. **Vegetables or fruit** take about half.
3. **Carbohydrate** takes the rest, scaled to your training that day.
4. **Fat** is the cooking oil, the nuts, the cheese. Measure it at first, it adds up fast.

Fiber and the micronutrient basics

Calories and protein get the attention, but a long series of reviews links **higher fiber and whole grain intake** with lower risk of heart disease, type 2 diabetes and colorectal cancer, and trials show lower body weight and cholesterol with higher fiber. The biggest benefit appeared at about **25 to 29 g of fiber per day**.[37] Most adults eat less. Beans, lentils, oats, vegetables, fruit, nuts and whole grains are the easy sources.

You do not need to count vitamins and minerals if you eat a varied diet with plenty of plants, some fish or eggs, and some dairy or fortified alternatives. Three cases deserve attention:

- **Vegan diets:** well planned vegetarian and vegan diets can be healthy, but need a reliable source of **vitamin B12** (fortified foods or a supplement), and attention to iron, calcium, iodine and omega-3.[38]
- **Heavy menstrual bleeding, frequent blood donation, or endurance training:** iron can run low. Test before supplementing, because too much iron is harmful.
- **Little sun exposure or darker skin at high latitudes:** vitamin D can run low. Test it (see Chapter 7).

For anything beyond that, a blood test and a registered dietitian will tell you more than any generic list.

CHAPTER 6

Meals and timing

How many meals

The idea that you must eat six small meals to "stoke your metabolism" is not supported. A meta-analysis of 15 studies first appeared to favor more frequent meals, but a sensitivity analysis showed the result came from a single study, so the authors urged caution.[20] Total intake matters, the number of meals does not.

So choose the pattern that keeps you full and fits your day. **Three to five meals,** each with a decent protein serving, is a sensible default. If you do better with fewer or more, use that.

Around training

The old idea of a 30-minute "anabolic window" after training has been revisited. A review concluded that the window is much wider than first claimed, and that **total daily protein matters more than the clock.**[21] A practical rule:

- **Before training (1 to 3 hours):** a meal with carbohydrate and protein you digest well.
- **After training:** a meal with protein and carbohydrate within a few hours. If you trained fasted, eat sooner.
- **During (over an hour or very hard):** carbohydrate and fluids if it helps you perform.[15]

Today

Choose your meal count and write down what your first meal and last meal will be tomorrow. Do not plan the whole week yet.

CHAPTER 7

Supplements: the short list

Most supplements are a waste of money. A few have decent evidence. None is a substitute for food, sleep or training. Here is how I think about them.

Supplement	Evidence	What to know
Creatine monohydrate	Strong. Among the most studied sports supplements.[22]	3 to 5 g per day, any time. No need to load or cycle. Small water weight gain is common. Ask a doctor first if you have kidney disease.
Protein powder	Strong, as a convenience.[10]	Not magic. It is food in a scoop that helps you reach your protein target.
Caffeine	Strong for performance.[23]	About 3 to 6 mg per kg about 60 minutes before training. Start at the low end. A common upper daily limit for adults is 400 mg.[24]
Vitamin D	Depends on your levels.	Test it. Intake above 4,000 IU per day is above the usual upper limit without medical supervision.[25]
Omega-3 (EPA/DHA)	Moderate. Most useful if you eat little fish.[26]	Two servings of fatty fish per week covers most people. High doses are a medical decision.

Everything else, including most pre-workouts, fat burners, "testosterone boosters" and BCAAs, either has weak evidence or is covered by getting enough total protein. If a product promises fast fat loss or muscle, assume it is selling you hope.

A note on safety

Supplements are regulated differently from medicines and can be contaminated. Choose products tested by a third party, and talk to your doctor if you take medication, are pregnant or have a medical condition.

CHAPTER 8

Alcohol and training

I have been sober for four years, and I do not tell people what to drink. I tell them what the research shows.

Alcohol is not nutritionally neutral for people who train. It contains about 7 kcal per gram, so drinks add up quickly. It also affects recovery: in a controlled crossover study, eight active men drank a large amount of alcohol (about 12 standard drinks) after hard training. **Even when they also ate protein, muscle protein synthesis fell by 24%, and by 37% when they ate carbohydrate instead.**[27] A large drinking session after training is a poor recovery strategy.

If you drink: keep it out of the post-training window, eat your protein first, and be honest about how much it adds to your weekly calories. If you are worried about how much you drink, or you cannot stop when you plan to, that is a conversation for a doctor, not a diet. Stopping suddenly after heavy use can be dangerous and should be done with medical guidance.

CHAPTER 9

Eating well with ADHD

A large meta-analysis of 42 studies (728,136 people) found that obesity was more common in adults with ADHD, with a pooled prevalence of 28.2% versus 16.4% in adults without ADHD (odds ratio 1.55). The researchers pointed to impulsivity and inattention as possible contributors, and the association held across study types.[28] The study does not prove cause, and it says nothing about you personally. But it explains something many of us know from experience: **the problem is rarely knowledge, it is execution.**

Here is what I do and what I tell my clients with ADHD. These are practical systems from my own experience and coaching, not trial results:

1. **Reduce decisions.** Eat the same breakfast and the same lunch most days. Variety is for dinner. Fewer decisions mean fewer chances to fall off.
2. **Make food visible and ready.** Wash and cut vegetables when you get home. Put protein where you see it first. Anything out of sight does not exist for an ADHD brain, good or bad.
3. **Set alarms for meals.** Hyperfocus can make you skip lunch and then overeat at night. A meal alarm is a tool, not a crutch.
4. **Keep a default meal.** Pick two or three meals you can make with your eyes closed (eggs and rice, yogurt and fruit, a tuna sandwich). When life gets messy, you are never starting from

zero.

5. **Remove the trigger foods from the house,** not from your life. It is far easier to not buy it than to resist it ten times a day.
6. **Track the minimum.** If logging everything feels impossible, log only protein for a month. That is a win.
7. **Plan around appetite changes.** If your medication blunts hunger, schedule a protein-rich meal before it kicks in and another when it wears off, and ask your prescriber about timing.

Ask your prescriber

Stimulant medication can suppress appetite and change when you feel hungry. If you take it, talk to your doctor about timing meals around it. Do not change medication because of a diet.

CHAPTER 10

Track, adjust, repeat

Plans do not fail on paper. They fail in the real week. The fix is a short feedback loop.

Research on self-monitoring finds a consistent link between tracking food and weight loss, although the quality of the evidence is mixed and most studies are observational.[29] The practical takeaway is that people who watch their numbers tend to do better, but you do not have to track everything forever.

A caution about tracking itself: people are not good at estimating what they eat. In a classic study of ten people who said they could not lose weight on less than 1,200 kcal a day, careful measurement showed their energy expenditure was normal. They had **under-reported their intake by about 47% and over-reported their physical activity by about 51%.**[33] This is not lying. It is normal human error, and it is the reason to weigh and measure your food for at least a few weeks.

The weekly check

1. **Weigh yourself** the same way (morning, after the bathroom) at least three days per week. Take the **weekly average.** Daily weight swings of 1 to 2 kg are normal.
2. **Compare** this week's average to last week's. Also use a waist measurement and how your clothes fit.

3. **Decide once per week,** never daily.

Result after 2 to 3 weeks	What to change
Weight is moving at about 0.5 to 1% per week	Nothing. Do not touch a working plan.
Weight not moving, and you followed the plan	Reduce intake by 100 to 200 kcal (mostly carbohydrate or fat), or add 2,000 steps per day.
Losing faster than 1% per week, feeling weak	Add 100 to 200 kcal. Protect muscle and training quality.
Did not follow the plan most days	Do not change the numbers. Simplify the plan (Chapter 9) and review Guide 1.

Notice the last row. If you did not follow the plan, the answer is almost never a stricter plan. It is an easier one.

Today

Write today's weight. Tomorrow, write it again. In seven days you will have your first weekly average.

CHAPTER 11

Plateaus and metabolic adaptation

Almost everyone who loses fat hits a slow patch. Understanding why keeps you from doing something drastic.

Your body defends its weight

The Minnesota Starvation Experiment, in which 36 healthy men were semi-starved for months, is still the clearest demonstration: most lost more than 25% of their weight, and many developed fatigue, irritability and extreme preoccupation with food.[30] You are not starving. But a smaller version of the same response happens in any diet. When you lose weight, your body tends to **burn less than predicted from your new size** and push up hunger. Researchers call it adaptive thermogenesis, and a review concluded that this opposition helps explain why most people regain what they lose.[31] The size of the effect varies between people and between studies, so do not read it as an excuse. Read it as a reason to be patient and consistent.

Small movements add up

In a classic overfeeding study, 16 volunteers ate 1,000 kcal per day above maintenance for eight weeks. Fat gain differed about tenfold between people, and the difference was largely explained by how much their **non-exercise activity** (fidgeting, posture, walking around) went up on its own.[32] On a diet the opposite happens: you move less without noticing. A daily step target is a simple way to protect it.

The plateau checklist

Check	Question
1. Time	Has it been at least 2 to 3 weeks, using weekly averages?
2. Accuracy	Are you weighing food, counting oils, drinks and bites, and logging weekends? (Remember the 47% finding.)
3. Activity	Have your daily steps dropped compared with the first weeks?
4. Sleep	Are you sleeping at least 7 hours? In one trial, dieters sleeping 5.5 hours lost 55% less of their weight as fat than when sleeping 8.5 hours.[34]
5. Stress and water	Stress, hard training, salt and the menstrual cycle all change water weight. Wait a week before reacting.
6. Duration	Have you been dieting for 12 or more weeks? Plan a break (Chapter 12).

Fix the cheapest item first. Only after all six, reduce calories by 100 to 200 kcal.

CHAPTER 12

Diet breaks and maintenance

Do not diet forever

A fat-loss phase should have an end. My rule of thumb in coaching is blocks of **8 to 16 weeks**, followed by time at maintenance. This is my practice, not a proven formula.

There is early evidence that planned breaks can help. In the MATADOR trial, 51 men with obesity dieted continuously for 16 weeks or in alternating 2-week blocks of dieting and maintenance, so that both groups spent the same time dieting. The intermittent group lost more weight (14.1 vs 9.1

kg) and more fat, with a smaller drop in metabolic rate after adjusting for body composition.[35] The catch: it took 30 weeks instead of 16, it was a small trial in men with obesity, and it may not apply to everyone. Use it as a reason to take breaks without guilt, not as a promise.

How to come out of a deficit

1. **Raise calories gradually,** by about 100 to 200 kcal per week, until your weight stops dropping. This is a common coaching practice, and I am not aware of strong trials showing that slow "reverse dieting" is better than going straight to maintenance, so choose what feels comfortable.
2. **Keep protein high and training steady.**
3. **Keep weighing yourself,** weekly average, and set a range (for example, plus or minus 2 kg).
4. **Have a plan for the first regain** (Chapter 10, weekly check).

What successful maintainers do

The US National Weight Control Registry followed 2,886 people who had lost at least 13.6 kg and kept it off for a year. Ten years later, **more than 87% were still maintaining at least a 10% weight loss.** Regain was associated with less physical activity, less self-weighing, less dietary restraint, more fat intake and more disinhibited eating.[36] It is observational and self-reported, but it is the best picture we have: successful maintainers keep the habits that got them there.

Today

Write your maintenance rules in one sentence each: how often I weigh, my weekly activity, and what I do when my weight goes above my range.

CHAPTER 13

Eating out, travel and social events

Real life does not happen in your kitchen. Here is how I handle it, from my own practice and coaching.

- **Decide before you arrive.** Look at the menu online. Pick the protein first, then add vegetables and a carbohydrate you enjoy.
- **Do not arrive starving.** Have a protein snack beforehand.

- **Eat slowly and stop at about 80% full.** Put the fork down between bites.
- **Share the dessert,** or have it and enjoy it. It is part of the 20%.
- **Bigger portions mean you eat more.** A Cochrane review found people consistently eat more when served larger portions, so ask for a box early or share a plate.[39]
- **One big meal is not a failed day.** Your next meal is a normal meal. Do not "make up for it" by skipping.
- **Travel:** pack protein bars or powder, fruit and nuts. Choose a place with a decent breakfast. Walk more than you normally would.
- **Alcohol:** decide the limit in advance, or pass (Chapter 8).

A sample day (about 165 g protein)

Meal	Example	Approx. protein
Breakfast	3 eggs, oats with milk and a banana	35 g
Lunch	150 g chicken, rice, vegetables, olive oil	50 g
Snack	Greek yogurt with berries, plus a scoop of whey	40 g
Dinner	150 g salmon, potatoes, big salad	37 g

Adjust the portions to your own numbers from Chapter 3 and 4. The point is the pattern: protein at every meal, plants at most meals, and the rest as you like.

CHAPTER 14

When to get professional help

This guide is for generally healthy adults. Please talk to a doctor or registered dietitian **before** changing your eating if any of these apply:

- You have a history of an eating disorder, or you notice rules around food that cause distress, binge eating, purging, or extreme restriction.
- You are pregnant, breastfeeding, or under 18.
- You have diabetes, kidney disease, heart disease, or any chronic condition, or take regular medication.
- You have lost weight very quickly without trying, or you feel dizzy, faint or unwell.

- You drink heavily and plan to stop.

Counting and tracking are tools, and for some people they make things worse. If tracking feels obsessive, stop and ask for help. That is strength, not failure.

CHAPTER 15

Pick your case: starter plans

If this is you...	Your starter plan
I want to lose fat	Calculate maintenance, subtract 300 to 500 kcal. Protein 1.8 to 2.2 g per kg. Fat about 25%. Rest carbohydrate. Weekly check. Use the 80/20 plate.
I want to gain muscle	Maintenance plus 200 to 300 kcal. Protein 1.6 to 2.0 g per kg. Train with resistance. Weekly check, aim for 0.25 to 0.5% of body weight per week.
I train hard (CrossFit, endurance)	Raise carbohydrate with training load (Chapter 4). Protein 1.6 to 2.0 g per kg. Eat before and after hard sessions.
I have ADHD	Chapter 9. Default meals, meal alarms, track protein only. Do not track everything on day one.
I yo-yo, binge, or hate being strict	Skip the calories. Do Chapter 5 only (80/20, no banned foods) plus a protein target for four weeks. See Chapter 14 if it feels out of control.
I am a beginner and overwhelmed	One change: protein at breakfast. Add one more change after 30 days. Read Guide 1, Chapter 9 ("one thing at a time").

APPENDIX

Worksheets

1. Your numbers

Step	Your answer
Weight (kg) / height (cm) / age	

BMR (Mifflin-St Jeor)	
Activity multiplier	
Maintenance (BMR x multiplier)	
Goal (deficit / maintain / surplus)	
Starting calories	
Protein (g) = weight x 1.6 to 2.2	
Fat (g) = calories x 25% / 9	
Carbohydrate (g) = remaining calories / 4	

2. Your default day

Meal	What I will eat	Protein (g)
Breakfast (default)		
Lunch (default)		
Snack		
Dinner (flexible)		

3. Weekly check (every Sunday)

Question	Answer
Average weight this week	
Average last week	
Days I hit my protein (out of 7)	
Training quality and energy (1 to 10)	
One change for next week	

About the author

Adriano Monteiro is a nutrition coach with Precision Nutrition certifications. He has lost 38 kg of fat, has competed in CrossFit for 8 years, has been sober for 4 years and has been in treatment for ADHD for 2 years. He writes and coaches for people who want lasting change without having to suffer in silence, and works with clients in English from Brazil.

Want help applying this to YOUR life?

Book a free 20-minute call. We will look at your numbers, pick your first change and build a plan that fits your real week. No pressure, no sales pitch, just a plan.

Book your free 20-minute call

https://calendly.com/adriano-nutrition/free-20-min-discovery-call

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